

ICD-0-3
 Carcinoma, urothelial NOS
 8120/3
 Site Bladder NOS C67.9
 JW 5/24/13

Operative Procedure:
 Radical cystectomy

Specimen Received:

- A: Left distal ureter (FS)
- B: Right distal ureter (FS)
- C: Bladder and prostate
- D: Right upper pelvic lymph nodes
- E: Right lower pelvic lymph nodes
- F: Left upper pelvic lymph nodes
- G: Left lower pelvic lymph nodes
- H: Left distal ureter; stitch=distal
- I: Right distal ureter; stitch=distal



Final Pathologic Diagnosis:

- A. Ureter, left distal, biopsy for frozen section diagnosis:
 Negative for malignancy
 Frozen section diagnosis confirmed
- B. Ureter, right distal, biopsy for frozen section diagnosis:
 Negative for malignancy
 Frozen section diagnosis confirmed

C. Urinary bladder, radical cystoprostatectomy:

Tumor histologic type: Urothelial carcinoma
 Variant histology (%): Not identified
 Tumor size: 5.0 cm
 Additional dimensions: 3.0 x 2.0 cm
 Grade (WHO 2004): High
 Tumor configuration: Invasive
 Associated epithelial lesions: Squamous metaplasia
 Microscopic tumor extension: Tumor invades perivesical tissue (microscopically)
 Lymph-vascular invasion: Present

Margins

Urethral: Free of neoplasm
 Ureteral: Free of neoplasm (frozen section diagnoses confirmed)
 Paravesical soft tissue: Free of neoplasm
 Other (specify): Not applicable

Additional pathologic findings: None

Paravesical lymph nodes: 0 positive of 1 lymph nodes

Criteria	Yes	No
Diagnosis Discrepancy		✓
Primary Tumor Site Discrepancy		✓
HIPAA Discrepancy		✓
Prior Malignancy History		✓
Dual/Synchronous Primary Noted		
Case is (check):	Q111/J1ED	D1511A/J111J
Reviewer Initials	JW	Date Reviewed: 5/18/13

All lymph nodes: 0 positive of 28 lymph nodes (parts C-G)
Size of largest lymph node metastasis: Not applicable
Extranodal extension (location): Not applicable

Pathologic stage (2010) pT3a pN0 pM-not applicable

Prostate, radical cystoprostatectomy:
Benign prostatic tissue with patchy atrophy
Negative for involvement by urothelial carcinoma

D. Lymph node (1), right upper pelvic, regional resection:
One lymph nodes with no pathologic change (0/1)

E. Lymph nodes (12), right lower pelvic, regional resection:
Twelve lymph nodes with no pathologic change (0/12)

F. Lymph node (1), left upper pelvic, regional resection:
One lymph nodes with no pathologic change (0/1)

G. Lymph nodes (13), left lower pelvic, regional resection:
Thirteen lymph nodes with no pathologic change (0/13)

H. Ureter, left distal, segmental resection:
Segment of benign ureter with mild chronic ureteritis
Negative for malignancy

I. Ureter, right distal, segmental resection:
Segment of benign ureter with mild chronic ureteritis
Negative for malignancy

The examination of this case material and the preparation of this report were performed by the staff pathologist.

Intraoperative Consult Diagnosis:

FSA: Left ureter:

No tumor in sections examined.
F/S TAT: 9 mins.

FSB: Right distal ureter:

No tumor in sections examined.
F/S TAT: 13 mins.

Gross Description:

The specimens are received in nine containers labeled with the patient's name,

Part A is received fresh for frozen section labeled "left distal ureter" and consists of a segment of ureter that has a length of 0.3 cm and a diameter of

0.4 cm, submitted in toto for frozen section evaluation. The residual frozen section tissue submitted in cassette A.

Part B is received fresh for frozen section labeled "right distal ureter" and consists of a segment of ureter that has a length of 0.3 cm and diameter up to 0.6 cm. The specimen is submitted in toto for frozen section evaluation. The residual frozen section tissue submitted in cassette B.

Part C is received fresh, subsequently placed in formalin labeled "bladder and prostate."

Specimen components and dimensions: The specimen consists of a urinary bladder (8.5 x 8 x 5 cm) with attached prostate gland (3.7 cm in length by 5.2 x 4 cm). The specimen is received previously inked and partially incised. The prostate gland is inked as follows: Right side yellow, left side black, posterior red. The anterior soft tissue edge is inked blue. The specimen is received with some associated perivesicular fibroadipose tissue.

Size, appearance, and location of tumor: There is a gray-tan to red-brown, focally ulcerative mass on the posterior bladder wall extending to the right lateral bladder wall near the area of the trigone that has overall dimensions of 5 x 3 by up to 2 cm. The cut surface of the mass is tan-white, firm with focal degeneration. This mass appears to approach the right ureteral orifice, however, the right ureter is probe patent.

Gross impression of invasive growth: The mass appears to be invasive into the bladder wall and focally abuts the right and left seminal vesicles.

Closest resection margins: The mass approaches the urethral margin within 3.2 cm, the posterior soft tissue edge within 0.7 cm, the right lateral soft tissue edge within 0.4 cm.

Lymph nodes attached: Sectioning through the perivesicular fibroadipose tissue reveals one possible lymph node, 0.6 cm in greatest dimension.

Other findings: The remaining bladder mucosa is pink-tan with the usual folds. Sectioning through the prostate gland reveals a pink-tan to white, firm cut surface.

Blocks submitted:

- C1-2 urethral margin (coned);
- C3 mass to posterior wall;
- C4 mass to posterior wall/right seminal vesicle;
- C5 mass to posterior wall/left seminal vesicle and trigone;
- C6 mass to right lateral bladder wall to inked soft tissue edge;
- C7 mass to right ureter;
- C8 left lateral bladder wall;
- C9 anterior bladder wall;
- C10 dome;
- C11 interface of mass to right prostate gland;
- C12-14 additional right posterior prostate gland;
- C15-16 right anterior prostate gland (apex to base);
- C17-20 left posterior prostate gland apex to base;
- C21-22 left anterior prostate gland apex to base;
- C23 one lymph node bisected.

Part D is received in formalin labeled "right upper pelvic lymph nodes" and consists of a portion of yellow-tan fibroadipose tissue, 3 x 2 x 1.2 cm. Sectioning reveals a single possible lymph node, 2.5 cm in greatest dimension. The lymph node is bisected and entirely submitted in cassettes D1 and D2.

Part E is received in formalin labeled "right lower pelvic lymph nodes" and consists of an aggregate of yellow-tan fibroadipose tissue, 6.5 x 6 by up to 2.5 cm. Sectioning reveals multiple lymph nodes up to 3 cm in greatest dimension.

These lymph nodes are entirely submitted as follows:

- E1-2 each with multiple whole lymph nodes;
- E3 one lymph node bisected;
- E4 one lymph node bisected;
- E5 one lymph node bisected;
- E6 one lymph node bisected;
- E7-8 one lymph node bisected (one half in each cassette).

Part F is received in formalin labeled "left upper pelvic lymph node" and consists of a single possible lymph node received with some associated fibroadipose tissue, 1.2 cm in greatest dimension. The lymph node is bisected and entirely submitted in cassette F.

Part G is received in formalin labeled "left lower pelvic lymph node" and consists of an aggregate of yellow-tan fibroadipose tissue, 5.8 x 4.5 x 1.5 cm. Sectioning reveals multiple lymph nodes up to 2.5 cm.

These lymph nodes are entirely submitted as follows:

- G1-2 each with multiple whole lymph nodes;
- G3-8 each with one lymph node bisected.

Part H is received in formalin labeled "left distal ureter, stitch distal" and consists of a segment of ureter received with some associated fibroadipose tissue that has a length of 7 cm and diameter up to 0.7 cm. There is a suture and clamp at one end, the suture designated distal. The sutured end is taken en face and submitted in cassette H1. The opposing end is taken en face and submitted in cassette H2. The remaining specimen is sectioned to reveal a central pinpoint lumen. Representative sections from the mid portion are submitted in cassettes H3 and H4.

Part I is received in formalin labeled "right distal ureter, stitch = distal" and consists of a segment of ureter received with some associated fibroadipose tissue that has a length of 3 cm and diameter up to 0.8 cm. There is a suture, as well as a metallic clamp at one end, designated distal. The distal end is taken en face and submitted in cassette I1. The opposing end is taken en face and submitted in cassette I2. The remaining specimen is sectioned to reveal a central pinpoint lumen. The remaining mid portion is submitted entirely sequentially from distal to proximal in cassettes I3 and I4.

Microscopic Description:

The final diagnosis of each specimen incorporates the microscopic examination findings.

END OF REPORT

Taken: DOB: (Age: , Gender: